CASE MANAGEMENT HANDOUT

LSH Case Management Training — Operational Conflict Resolution: Intake, Planning & Treatment

Worked examples: *John Doe v. Robert Smith* · LSH-2026-MVA-770412 · MVA, passenger injury · Lead Counsel Atty. Elijah Thone

This comprehensive operational handout provides Case Managers with actionable, step-by-step methodologies to identify, manage, and resolve critical conflicts across the Intake, Case Planning, and Active Treatment phases of a Personal Injury claim.

Proper mastery of these guardrails guarantees steady file progression, structural legal compliance, and bulletproof defense frameworks against insurance mitigation strategies.

1. Intake Phase: Conflict Identification & Gatekeeping

The Intake Phase serves as the primary quality control filter for the firm. Operational bottlenecks, structural informational deficits, and legal exposure must be systematically neutralized before a file is approved for formal case opening.

A. Core Trajectory Mapping & Conflict Resolution

- **Step 1: Execute System Matrix Setup** - Open your new file profile simultaneously in the Case Management System (CMS/Casepeer) and the firm's centralized digital storage repository (Sharepoint). Input your profile name into the dedicated 'Lien Negotiator' field to assert absolute administrative ownership.
- **Step 2: Run the Mandatory Conflict Check** - Before recording detailed narrative disclosures from the client, cross-examine the primary intake data against the internal firm index. Query the full legal names of the prospective client, all adverse operators, the host driver and registered vehicle owners, resident relatives, known eyewitnesses, and every insurance carrier on the file. In *John Doe v. Robert Smith* that means: John Doe (client), Robert Smith (adverse driver), Kevin Alvarez (host driver and owner of the 2020 Ford Escape), Margaret Doe (John's mother, resident relative), Ruth Calloway (independent witness), and the carriers Keystone Mutual Insurance, Harbor Point Insurance, Cardinal State Mutual and Bluewater Auto Insurance. If an exact name match or direct familial/business relationship appears inside an active, open, or historical file, instantly freeze the intake data entries. Do not log confidential facts. Check the box marked 'Escalated for Attorney/Senior Review' and route an urgent advisory notice to the handling attorney to clear ethics compliance.

B. Common Bottlenecks & Operational Fixes

- **Issue 1: Incomplete Primary Information or Missing Signatures** - The prospective client submits a vague narrative description of the Motor Vehicle Accident (MVA) and fails to execute authorization forms or electronic retainers.
  - **Operational Fix:** Do not let the file languish in an indeterminate status. Deploy an automated 3-day notification sequence through the CMS. If the client remains unresponsive to clarification requests after 72 hours, execute a structured verification follow-up call. If contact fails, change the file status to 'Declined due to Eligibility/Risk Factors' to protect firm resources.
- **Issue 2: Ambiguous Liability Profiles** - The initial facts of loss reveal an intersection collision where the right-of-way assignment is unclear, or the police report has been delayed by municipal agencies.

- **Operational Fix:** Do not make unilateral case-opening decisions on unconfirmed liabilities. Categorize the profile as 'Escalated for Attorney/Senior Review'. Task a dedicated assistant to track down the unredacted accident exchange details, and preserve a clean internal case log detailing the precise open liability questions requiring legal review.

2. Case Planning Phase: Coverage & Liability Projections

Every newly accepted case file must undergo a comprehensive tri-panel assessment within the first 30 days to determine its maximum valuation and operational path.

A. The Regulatory Triple-Check

- **Step 1: HIPAA Compliance & Specialized Authorizations** - Secure an explicit, claim-specific Authorization to Release Medical Information signed by the client. The form must specifically include the exact Date of Loss (DOL) and grant the firm permission to interact with legal representatives, medical billing clearinghouses, and first-party auto insurance adjusters.
- **Step 2: Medicare/Medicaid Federal Subrogation Disclosure** - Analyze the intake demographics to confirm if the client is aged 65 or older, holds active Medicare benefits, or receives Social Security Disability Insurance (SSDI). If federal health parameters apply, you must notify the Centers for Medicare & Medicaid Services (CMS) or the Benefits Coordination & Recovery Center (BCRC) immediately. Failure to establish an official subrogation case profile early can result in permanent settlement freezes and structural federal penalties.
- **Step 3: Map the Primary Payor Order Hierarchy** - Establish the exact coordination of benefits sequence before authorizing clinical evaluations. The standard structural payment roadmap must follow this strict tier: Tier 1: Personal Injury Protection (PIP) / MedPay → Tier 2: Private Health Insurance / ERISA Plan → Tier 3: Letter of Protection (LOP) / Medical Liens.

B. Analyzing Layered-Coverage Claims (e.g., John Doe v. Robert Smith)

When the at-fault driver carries a low liability limit, your case plan must adjust to map every layer of coverage the client can reach:

- **Coverage Limits:** Robert Smith’s carrier, Keystone Mutual, carries only $10,000 in bodily injury (BI) coverage per person. Behind it stand two first-party uninsured/underinsured motorist (UM) layers: Host UM on Kevin Alvarez’s Harbor Point policy ($20,000) and Relative UM on Margaret Doe’s Cardinal State Mutual policy ($10,000), a $40,000 stack. The order is fixed: BI → Host UM → Relative UM. UM opens only after BI is exhausted (paid or tendered at limits), so a UM demand before the BI tender is premature. Before any BI settlement or release, the UM carriers’ written consent must be in the file; releasing BI without it can forfeit the stack.
- **Why Passenger Status Matters:** John was the front-seat passenger in a car he does not own. That position is what reaches Host UM (as an occupant of the host vehicle) and Relative UM (as a resident relative in a non-owned vehicle). Had he been driving his own 2019 Honda Civic, he would reach $10,000: he rejected UM on his Bluewater policy in writing, and Cardinal State’s policy excludes a resident relative occupying a vehicle he owns that it doesn’t insure. Record the client’s seat position precisely at intake.
- **Liability Anchors:** At the all-way stop at Maple Hollow Rd & Birch Ave, Smith did not stop and struck the Escape’s front passenger door. Riverton PD report RPD-26-018842 cites him for failure to stop at a stop sign; an independent witness, Ruth Calloway, gave a signed statement on scene; and Margaret Doe’s doorbell-

camera video shows the Escape stopped and Smith rolling through. Preserve all three early. Adjusters will still search for comparative negligence elements to mitigate their exposure.

- **The Action Plan:** You cannot project a case accurately without validating liability and limits. Order the official police report immediately, and get every limit verified in writing (declarations pages) early: Keystone would not disclose its limit until a written request with authorizations, and its declarations page confirmed the $10,000 BI limit on 03/20/2026. If liability is contested, a limit is withheld, or the BI carrier stalls and freezes the stack, write up the open questions with your recommendation and route the file to the handling attorney, Atty. Thone, who decides whether to file suit. Do not let the client's treatment timeline run on uncooperative adjusters.

3. Treatment Phase: Monitoring Gaps, Red Flags, & Crises

The Treatment Phase is the operational core of case management. Your primary focus is maintaining structural file integrity to defeat insurance utilization reviews.

A. Active Treatment Gap Intervention Protocols (A-C-T)

Insurance defense software scans medical packages to identify breaks in treatment timelines. These gaps are used to argue that the client's injuries are either resolved or unrelated to the accident. Common causes include missed clinical appointments, sudden transportation failures, or personal crises.

- **Assess:** Monitor weekly provider attendance reports. If a client misses consecutive appointments or displays a 14-day gap in active care, flag the file immediately, then contact the client directly and conduct a structured interview to discover the root cause.
- **Coordinate:** Remove the barrier you found: work with the provider, the carrier and the client's household on transportation, scheduling or authorization, and confirm the new plan with the client.
- **Track:** Record the client's specific explanation, every coordination attempt and its result, and the date care resumed in the CMS notes. This preserves an unalterable audit trail that your legal team can use to neutralize defense challenges during negotiations.

B. Case Manager Master Escalation Ladder

When an administrative, clinical, or insurance obstacle blocks the standard treatment roadmap, apply this progressive framework:

- **Level 1 (Clinical):** Contact the provider's office manager directly to secure delayed treatment records or coordinate an expedited specialist consultation.
- **Level 2 (Administrative):** Issue an internal 'Notice of Delay' to document external care barriers (such as delayed insurer authorizations for an MRI scan).
- **Level 3 (Legal):** Request the handling attorney to file a formal motion or execute an explicit Letter of Protection (LOP) to bypass insurance stalling tactics completely.

4. Clinical Simulation Workbook

Review these two common real-world operational scenarios to master advanced case tracking techniques.

CASE STUDY 1: The 'Eggshell Plaintiff' Core Strategy

- **The Conflict:** Your client, John Doe, suffered a C5-C6 right paracentral disc herniation with mild right foraminal narrowing when Robert Smith failed to stop at the all-way stop and struck the Escape's front passenger door. The adverse carrier (Keystone Mutual) calls the MRI findings "degenerative." It points to chiropractic visits John had in 2021 and argues his current pain comes from a pre-existing condition.
- **The Operational Solution:** Do not allow the adjuster to label this a simple soft-tissue or degenerative case. Your job is to gather and organize the records that answer the argument. The medical conclusions belong to the physician and the legal argument to the attorney. Use the following step-by-step procedure:
  - **Step 1:** Secure the 2021 records from Birch Avenue Chiropractic (Dr. Lena Ford, DC) to build a clear clinical baseline: three visits (05/04, 05/11 and 05/18/2021) for low-back stiffness, resolved, released to full activity. No prior neck complaints.
  - **Step 2:** Secure the post-accident MRI report and images: the cervical MRI at Summit Imaging Center on 03/13/2026, read by Dr. Helen Park, shows the C5-C6 herniation and "no significant degenerative change at the adjacent levels."
  - **Step 3:** Compare the pre- and post-accident records side-by-side. The 2021 visits concern a different body region (the low back, not the neck), and the radiologist found the adjacent cervical levels clean. Lay these out as an organized comparison of what the records say, not as your own medical opinion.
  - **Step 4:** Request the treating physician's causation letter. Dr. Alana Reyes (Lakeside Orthopedics) wrote on 06/19/2026 that the injury was more likely than not caused by the collision.
  - **Step 5:** Frame the package for the attorney. Deliver the baseline records, the MRI report, the side-by-side comparison and the causation letter to Atty. Thone in a short memo. Whether and how to argue the Eggshell Plaintiff Doctrine or causation to the carrier is his decision; once he approves a rebuttal, you build and send it.

CASE STUDY 2: The 'Transportation Wall' Crisis Resolution

- **The Scenario:** Your client has been compliant with his chiropractic and physical therapy program at Riverton Spine & Rehab since 02/24/2026. Then his transportation falls apart: Keystone ended his rental car on 03/19/2026 at the 30-day cap while his Civic sat waiting for parts, and on 04/01/2026 his coworker's shift changed, ending their carpool. He misses four visits (04/03, 04/07, 04/09 and 04/14). You learn of it on 04/08/2026 from the provider's missed-visit alert.
- **The Operational Risk:** If the client stops treatment now, a massive treatment gap will form on his file, on top of the 14-day gap already there before his first treatment. The insurance company will argue that his injuries have resolved, destroying the settlement value of the claim.
  - **Step 1: De-escalate and Educate** Call the client the same day (04/08). Validate his stress, and explain why steady care matters to his file and how stopping treatment can be used by the insurer to deny the severity of his injuries. Route any legal question he asks to the attorney.
  - **Step 2: Coordinate Alternative Transport Solutions** Check which first-party benefits might cover rideshare or medical transport services. Ask whether the carrier that ended the rental will extend it (here, Keystone refused on 04/10). Contact the treating clinic; ask about shuttle pick-up programs or schedule changes. On this file the fix was moving John to evening slots so his mother, Margaret, could drive him after her shift; the first open evening slot was 04/21. If local transport options are

unavailable, check if the clinic can arrange a temporary transition to home-based physical therapy or a remote telehealth monitoring program to keep the treatment timeline active.

- **Step 3: Implement an LOP Funding Pivot** If alternative transportation is unavailable, consult with the handling attorney to see if a portion of the case budget can be allocated toward a medical transport account through a letter of protection with a specialized funding company.
- **Step 4: Record the Modification** Log the exact dates of the transportation failure (03/19 and 04/01), the missed visits, the client's explanation, and your corrective steps into the CMS notes, and confirm attendance once care resumes (here, 04/21 and 04/23). This is the A-C-T protocol (Assess, Coordinate, Track); it ensures that a disruption in care (19 days with no visits, 04/02 → 04/21) is clearly documented as a transportation issue rather than a recovery or clinical plateau.

5. Revenue & Monitoring Compliance Checklist

Before marking any case file as demand-ready, you must verify that all firm cost records are completely balanced:

Pre-Litigation Investigation Fee Schedule (Add123 Platform)

- Comprehensive Driver's License History Query: $3.42 (Historical archive rate prior to 3/14/24: $1.89)
- Registered VIN / Title History Search: $3.42 (Historical archive rate prior to 3/14/24: $1.74)
- Tag and License Plate Registration Search: $1.74
- Real-Time State Insurance Coverage Query: $3.00

Certified Outbound Document Postal Schedules

- Standard First Class Postal Rate: $0.69 (Historical Jan 2024 scale: $0.64)
- Secure Priority Delivery Service: $8.50
- Certified Mail with Return Receipt Requested: $8.69
- Priority Mail with Signature Confirmation (Settlement Checks Only): $12.20 (Historical Jan 2024 scale: $12.00)

Clinical Monitoring Standards & 'Red Flag' Offer Caps

Diagnostic Category	Objective Monitoring Metric	Maximum Settlement Offer Cap
Chiropractic Care	Documented progress on the Oswestry Disability Index (ODI) or Visual Analog Scale (VAS) within 6 weeks.	$2,500 Max Offer if PIP paid $8k+; Full Waiver requested if PIP hit the $10k cap (Max offer capped at $1,000 if waiver is denied).
MRI Scan Views	Standalone facility imaging interpretations and matching independent billing ledgers.	$800 - $1,000 per standard view (Brain MRI capped at $1,100) assuming zero PIP coverage. Capped at $225 if primary PIP fee rules apply.
Pain Management Injections	Operative reports, anesthesia charts, and device/implant invoices.	Flat $1,500 maximum cap per procedural date for Epidural Steroid Injections, Facet Blocks, or MBBs, assuming zero PIP coverage.
Specialist Consults	Documented Date of Loss (DOL) confirmation on specialist medical reports.	Initial Orthopedic MD evaluation capped at $300; subsequent follow-up clinical evaluations limited to $150.